

pancreatic-recurrent-kras-g12r-m8f3

Plain-language summary for patient and caregiver

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What this page is

You asked a focused question: in pancreatic cancer that came back after standard adjuvant chemotherapy, and that carries a KRAS G12R mutation along with a few other findings, what targeted options are worth talking about? This page is the patient-facing answer. It walks through the therapeutic options in rough order of how strong the case for each one is, with the workup tests that unlock some of them called out where they belong. The clinician version of this page covers the same ground with the technical detail your oncologist will want.

One thing to settle up front, because it comes up a lot. You may have seen headlines about "KRAS drugs" that now exist as a pill in lung cancer. Those apply to one specific KRAS variant called G12C. Your cancer has a different variant, G12R, and the lung-cancer drugs (sotorasib and adagrasib) do not bind it. The newer class that does reach G12R is still in trials. That is why most of what follows is trial enrollment rather than a prescription you fill at a pharmacy.

And one boundary. This page only covers drugs aimed at the specific features in your tumor profile. The standard chemotherapy regimens your oncologist already uses for pancreatic cancer are not listed here, and that is not a judgment about them. The question was about targeted options; the chemo conversation is a separate one that stays with your treating team.

What we know about your cancer

You are in your fifties. The cancer started in your pancreas and is the most common type, adenocarcinoma. It came back after surgery and after adjuvant chemotherapy. That adjuvant regimen was FOLFIRI, which is built around a drug called irinotecan, and the cancer grew despite it. One consequence matters below: you have not yet had a platinum chemotherapy (oxaliplatin or cisplatin). You are up and about day to day with somewhat less energy than usual, which clinicians score as ECOG 1. You are not on any cancer treatment right now.

The tumor's molecular profile shows several findings, all confirmed by sequencing:

- A **KRAS G12R** mutation. KRAS is a gene that, once mutated, jams a cancer cell into "grow" mode. G12R is one particular flavor of that mutation, and it turns up in roughly one in five pancreatic cancers. It is the main driver here and the target most of the options aim at.
- An inactivating mutation in **TP53**, the most commonly damaged tumor-suppressor gene in cancer. This is background context, not a direct treatment target.
- Loss of **CDKN2A** and an alteration in **CCND3**. Both feed the same machinery that controls how fast cells divide, which is why one option below tries to block that machinery.
- The tumor is **MSS** with a low mutation count (a marker called TMB, measured at 4.1). In plain terms, this cancer does not carry the signals that make immunotherapy drugs like pembrolizumab work on their own, so those are off the table here.

What you told us matters most

You said the goal is remission, and you are willing to accept high-risk, high-reward options to chase it. You would rather keep side effects down where that is possible, but you set no hard limits. There is no treatment you ruled out in advance because of its side effects. You lean toward clinical trials. Put together, that points toward the options with the deepest possible upside, accepting more uncertainty and more toxicity than a more cautious

patient might. The ordering below reflects that.

The options

A note before the list. Almost everything here is investigational, meaning drugs being tested in trials rather than approved treatments for your situation. The single drug on this page that guidelines actually name (olaparib) is one you cannot start today, for reasons explained under its heading. So the ranking is less "proven versus unproven" and more "which promising-but-early option has the most behind it." Where a number can be given honestly, it is; where it cannot, that is said plainly rather than dressed up.

Option 1 — daraxonrasib (RMC-6236)

This is the option to discuss first, and it is the one every part of the review agreed on. Daraxonrasib is an oral once-daily pill designed to shut down mutant KRAS across several variants, including G12R. It is the furthest along of anything that targets your main driver.

Here is the honest shape of the evidence. In a published early trial, second-line pancreatic cancer patients with a KRAS G12 mutation who took the 300 mg dose (26 patients) had their tumors shrink about 35% of the time, and lived a median of roughly 13 months. For this setting, a 35% response rate is about double what standard second-line chemo tends to deliver, so the chance of a meaningful response is the best on this page. Two honest discounts, though. First, that 35% pools together several KRAS G12 variants. Your specific G12R was not reported on its own, so the exact number for G12R is genuinely uncertain. Second, G12R behaves a little differently from its cousins in the lab, and no one yet knows whether that helps, hurts, or makes no difference.

You may also come across news that a larger phase-3 trial (RASolute 302) reported a survival benefit, and that the FDA has opened an expanded-access route. Those figures come from the company and the FDA, not from a published, peer-reviewed study, so they are worth knowing as context but should not be treated as settled proof. The strength of this option rests on the published trial described above, not on the press figures.

The side effects are real but mapped out. Nearly everyone gets some treatment-related effect; about 3 in 10 get a serious (grade 3 or higher) one. The most common is an acne-like rash (very common, occasionally severe), along with diarrhea, nausea, mouth soreness, and fatigue. None of this crosses a line you drew, and importantly, each of these has a written management plan: creams and a low-dose antibiotic for the rash, anti-diarrhea medication, a medicated mouthwash for the sores. There were no treatment-related deaths in the trial.

This fits what you told us well: an active trial you can enroll in, an oral pill, the deepest published upside on your main target, and no side effect that trips a limit you set. What is contested is narrow and specific: the exact benefit for G12R on its own is not yet published, and the phase-3 survival figures are not yet peer-reviewed. Practically, enrollment may run through the monotherapy trial, a combination-platform trial, or the expanded-access route; your team would confirm which door is open.

Option 2 — ERAS-0015 (a pan-RAS "molecular glue")

This option carries caveats. Here is the tradeoff to weigh. ERAS-0015 is another oral once-daily drug that goes after mutant KRAS, using a different mechanism (it acts like a "glue" that traps the KRAS protein). Early results look striking: in a small group of pancreatic cancer patients, tumors shrank around 40% of the time, and every one of 14 patients with a KRAS G12 mutation showed a large drop in tumor DNA circulating in the blood, with five

clearing it entirely.

The caveat is about how ripe that evidence is. Those numbers come from a single company announcement in April 2026, not a published paper or even a conference abstract. The responses were measured early (at 14 weeks) and are not yet confirmed. And a critical piece is simply missing: the company has not released how often serious side effects happened, broken down by type. Without that table, there is no way to hand your care team a plan for managing what might go wrong, which is the main reason this sits below Option 1 rather than beside it. One more honest note: the blood-DNA drops are encouraging, but in pancreatic cancer that particular signal has never been shown to reliably translate into tumors actually shrinking. So treat the promise here as real but unproven.

Why it is on the page at all: it is a second, independent shot at your main target with an upside in the same range as Option 1, and your stated appetite for high-risk, high-reward trials is exactly what it fits. The open question is maturity. One option is a published study, the other is a headline. If a full dataset with a safety breakdown comes out, this could move up quickly.

Option 3 — olaparib (a maintenance pill, only after specific conditions are met)

This option carries caveats, and the biggest one is timing: it cannot start today. Olaparib is the one drug on this page that treatment guidelines actually name for pancreatic cancer, but it only applies to a specific situation, and two things have to line up first.

First, olaparib only works in people whose cancer carries an inherited BRCA1 or BRCA2 mutation (a related gene, PALB2, counts too). That requires a germline (inherited) genetic test that has not been done yet. Second, olaparib is a *maintenance* drug: it is meant to hold the line after a course of platinum chemotherapy has already controlled the disease, and you have never had platinum chemo. So even in the best case, this is months away, not now.

If both conditions are met, the benefit is real: in the trial that established it, patients on olaparib went about twice as long before their cancer grew again (roughly 7 versus 4 months) compared with placebo. Two honest tempering points. That delay-in-growth benefit did not translate into people clearly living longer in the trial; the survival curves stayed close together. And until the genetic test comes back positive and a platinum course holds the disease, you should assume no benefit here, because the drug simply does not apply yet. Side effects include anemia, fatigue, and nausea (serious in about 4 in 10 patients), plus a small long-term risk (around 1.5%) of a serious blood disorder with prolonged use.

What is contested is not the drug's mechanism but its deliverability for you right now. An option double-gated on a test you have not had and a chemotherapy you have not received is hard to rank as an immediate move for someone whose goal is remission now. The concrete action it points to is worth taking regardless: **ask about ordering the germline BRCA1/2/PALB2 panel now**. Guidelines recommend it for every pancreatic cancer patient regardless of family history, and a positive result would open this pathway (and Option 9).

Option 4 — daraxonrasib plus chemotherapy

This option carries caveats. This is Option 1's drug combined with a standard chemo backbone (either mFOLFIRINOX or gemcitabine plus nab-paclitaxel), taken in a trial. The logic fits a remission-first goal: because your adjuvant chemo was irinotecan-based, you have never had oxaliplatin, which keeps the intensive mFOLFIRINOX combination biologically on the table. In a very small early cohort on a related combination, 5 of 6 patients had confirmed tumor shrinkage. That is a striking signal, but from just six people, so it cannot carry much

weight yet.

The tradeoff is toxicity stacked on toxicity. You would be layering the rash and gut side effects of the KRAS drug on top of chemo's blood-count drops and nerve effects, and there is no dedicated safety readout for this specific combination in pancreatic cancer. If lingering nerve symptoms from your earlier FOLFIRI are a factor, the gentler gemcitabine backbone may be the better choice. This sits below the single-drug version of daraxonrasib precisely because the combination's safety is less charted, and the practical question is whether the pancreatic arm is open at your referral site.

Option 5 — ulixertinib plus palbociclib

This option carries caveats. This pairing is the only one on the page that goes after three of your tumor's features at once: it blocks a signal (ERK) downstream of KRAS, and it targets the cell-division machinery tied to your CDKN2A loss and CCND3 alteration. On paper the logic is clean.

The honest limit is the clinical track record. The best published result in pancreatic cancer was stable disease (tumors held steady rather than shrank), not actual shrinkage. And one of the two drugs (palbociclib) has already been tested on its own in cancers with a related cell-division marker and largely failed there. The lab work behind the combination was mostly done on other KRAS variants, not G12R, so whether it applies cleanly to your cancer is untested. Side effects include overlapping drops in blood counts, fatigue (common), rash, and nausea. It is on the page as a deliberate "covers the most features" option, not because the response numbers are strong. The mechanistic case is the appeal, and the response expectation is modest.

Option 6 — KST-6051 (FALCON trial)

This option carries caveats. This is another oral drug against mutant KRAS, built to work on the protein in more than one of its states, with pancreatic cancer named as a target. The plain truth is that there is no data yet. The first patient was dosed in April 2026, so no one can tell you how well it works or what its side effects are. It is on the page as a forward-looking backup: if the more-established KRAS drugs stop working or the rash proves intolerable, this is a mechanistically distinct shot at the same target. Being this early, trial slots will be scarce, and Option 8 is a near-identical alternative.

Option 7 — PF-07934040

This option carries caveats. Another oral pan-KRAS drug in its first human trial. What sets it slightly apart is that it names G12R explicitly in its written eligibility criteria, and it has a second-line pancreatic cohort that matches your setting. But like Option 6, it is early enough that no results on how well it works have been reported, and its side-effect picture is limited to what the protocol excludes. It is a clean backup on your main target with no efficacy readout yet. One practical note: the trial excludes people with significant nerve damage, which is worth checking against any lingering effects from your earlier chemo, though irinotecan-based regimens usually spare the nerves.

Option 8 — JAB-23E73

This option carries caveats. A second early pan-KRAS drug held on the list for redundancy: if one early trial has no open slot, another might. Its lab profile covers G12R, and it is open to patients like you who have not had a prior KRAS-targeting drug. Two honest limits: the only public hint of activity so far is a vague "early antitumor activity" statement with no numbers, and the trial enrolls any advanced solid tumor rather than naming pancreatic

cancer, so a pancreatic slot is not guaranteed and would need confirming.

Option 9 — rucaparib (the maintenance backup to olaparib)

This option carries caveats. Rucaparib is in the same family as olaparib (Option 3) and comes with the same two gates: it needs a BRCA1/2 or PALB2 mutation (inherited or in the tumor) and a prior platinum chemotherapy course. Its one advantage is that it covers a slightly wider set of genetic results than olaparib does. In its trial, patients with the right genetic makeup did well (tumors shrank about 42% of the time, with a median of roughly 13 months before growth). But the evidence is from a smaller, single-group study rather than a rigorous head-to-head trial, and the same "cannot start today" gate applies. The same germline panel that unlocks olaparib unlocks this. It sits lower on the list because it depends on a marker outside the main targetable features this page is scoped to.

Option 10 — anvemetostat (needs an extra test first)

This option carries caveats. This drug targets a vulnerability that can come with your CDKN2A loss. A neighboring gene called MTAP is often deleted alongside CDKN2A, and when it is, this class of drug can exploit that. There is a version of the trial that pairs it with daraxonrasib, which would cover both your KRAS and cell-cycle features at once. Two big caveats. First, no pancreatic cancer results have been published, and there is no safety data on file. Second, and this is the gate: the whole rationale depends on MTAP actually being deleted, which has not been tested. **Ask about running the MTAP reflex test** (a straightforward add-on to existing tissue analysis). If MTAP turns out to be intact, this option falls away entirely.

Option 11 — ELI-002 (a KRAS vaccine)

This option was considered but not recommended for your situation, though it stays on the page because it is worth understanding why. ELI-002 is a vaccine that trains the immune system against the exact G12R mutation your cancer carries, and its safety record is the cleanest of anything here (essentially injection-site reactions, nothing serious attributed to it). In a trial, 84% of patients mounted the intended immune response.

The problem is the setting. Every human result for this vaccine comes from patients who had their cancer surgically removed and had only tiny, invisible traces of disease left, not patients with visible, active recurrence like yours. There is no evidence a vaccine can control bulk recurrent pancreatic cancer, and the registered successor trial is not currently enrolling. So the mechanism is elegant and the safety is excellent, but the window it was studied in does not match where you are. That mismatch, not any safety worry, is why it lands at the bottom.

Questions to ask your oncologist

- Can we order the germline BRCA1/2/PALB2 panel now, and can we add the MTAP reflex test to the tumor analysis? Both are low-risk tests that decide whether some options on this list are even possible.
- If the germline test comes back **negative**, what does that change? (It would close the olaparib and rucaparib pathways, so what would we focus on instead?)
- For Option 1 (daraxonrasib): is there an open trial slot for me, or an expanded-access route, and what is the plan for monitoring and managing the rash and diarrhea if I start?
- For Option 2 (ERAS-0015): given that the serious-side-effect data has not been released, how comfortable are you enrolling me, and would you want to wait for a fuller safety readout?
- For Option 4 (daraxonrasib plus chemo): does my earlier FOLFIRI leave me with any lingering nerve symptoms

that would steer us toward the gentler chemo backbone?

- If I were to consider olaparib later, would you recommend a platinum-based chemo course now regardless, and what would tell us it is working well enough to move to maintenance?
- Several of these are very early trials with no results yet. How do you weigh enrolling in something unproven against options with more data behind them, given that my goal is remission?
- Is there a single trial that could cover more than one of my tumor's features at once, and would you prioritize that?

Sources

Published studies (PubMed IDs): 42090791, 38589574, 38593348, 31157963, 33970687, 39437014, 40790272, 38195752, 31649109, 37552839, 26912361, 26912360

Trials (ClinicalTrials.gov IDs): NCT05379985, NCT06445062, NCT06625320, NCT06895031, NCT07458347, NCT06959615, NCT06447662, NCT03454035, NCT06360354, NCT05726864